QLABS DIAGNOSTICS

Patient ID	: QLD24/1917	Sample ID	: 2407800
Name	**: MR MAHESH**	Registration Date	: 18/07/2024 08:00 AM
Age/Gender	: 35 Yrs/Male	Report Date	: 18/07/2024 11:38 AM
Ref. Dr.	: DR SANDEEP REDDY	QLD24/1917	

HAEMATOLOGY REPORT

Test Description	Result	Unit	Biological Reference Ranges
COMPLETE BLOOD PICTURE			
Hemoglobin	**12.0**	gm/dL	13.0 - 17.0
RBC Count	**3.81**	mil/cu.mm	4.0 - 5.5
Hematocrit HCT	**34.5**	%	40 - 54
Platelet Count	**0.46**	lac/cmm	1.5 - 4.5
Red Cell Indices			
Mean Corp Volume MCV	90.6	fL	83 - 101
Mean Corp Hb MCH	31.5	pg	27 - 32
Mean Corp Hb Conc MCHC	**34.8**	gm/dL	31.5 - 34.5
RDW-CV	12.9	%	11.6 - 14.0
Total Count and DIFFERENTIAL LEUCOCYTE COUNT			
Total WBC Count	**1950**	cell/cu.mm	4000 - 10000
Neutrophils	43	%	40 - 75
Lymphocytes	**47**	%	20 - 40
Eosinophils	04	%	01 - 06
Monocytes	06	%	02 - 10
Basophils	00	%	00 - 02
Microscopic Blood Picture			
RBC Morphology	**Anisocytosis Normocytic Normochromic Anemia**		
WBC Morphology	**Leucopenia With Few React Lymphocytes Seen**		
Platelet on smear	**severe Thrombocytopenia**		

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BIOCHEMISTRY REPORT

Test Description	Result	Unit	Biological Reference Ranges
LIVER FUNCTION TEST (LFT)			
TOTAL BILIRUBIN *Method: Serum, Jendrassik Grof*	**3.10**	mg/dl	0.2 - 1.2
DIRECT BILIRUBIN *Method: Serum, Diazotization*	**1.2**	mg/dL	0.0 - 0.3
INDIRECT BILIRUBIN *Method: Serum, Calculated*	**1.90**	mg/dl	0.0 - 1.0
SGPT (ALT) *Method: Serum, UV with P5P, IFCC 37 degree*	**69**	U/L	0 - 49
SGOT (AST) *Method: Serum, UV with P5P, IFCC 37 degree*	**221**	U/L	0 - 46
ALKALINE PHOSPHATASE *Method: DGKC*	78	U/L	53 - 130
TOTAL PROTEIN *Method: Serum, Biuret, reagent blank end point*	6.40	g/dl	6.0 - 8.0
SERUM ALBUMIN *Method: Serum, Bromocresol green*	3.22	g/dl	3.2 - 4.6
SERUM GLOBULIN *Method: Serum, Calculated*	3.18	g/dl	1.8 - 3.6
A/G RATIO *Method: Serum, Calculated*	**1.01**		1.2 - 2.2

NOTE :

In known cases of Chronic Liver disease due to Viral Hepatitis B & C, Alcoholic liver disease or Non alcoholic fatty liver disease, Enhanced liver fibrosis (ELF) test may be used to evaluate liver fibrosis.

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Amylase-serum	**124**	U/L	10 - 90

Method: Enzymatic

Clinical Significance

Amylase - The major sources of amylase are the pancreas and the salivary glands. The most common cause of elevation of serum amylase is inflammation of the pancreas (pancreatitis). In acute pancreatitis, serum amylase begins to rise within 6-24 hours, remains elevated for a few days and returns to normal in 3-7 days. Other causes of elevated serum amylase are inflammation of salivary glands (mumps), biliary tract disease and bowel obstruction. Elevated serum amylase can also be seen with drugs (e.g., morphine) which constrict the pancreatic duct sphincter preventing excretion of amylase into the intestine.

Lipase-Serum	28	U/L	5-31

Method: Methy resourufin ester

COMMENTS:

Lipase measurements are used in the diagnosis and treatment of diseases of the pancreas, such as acute pancreatitis, chronic pancreatitis, and obstruction of the pancreatic duct. In acute pancreatitis serum lipase activity tends to become elevated & remains for about 7 - 10 days .Increased lipase activity rarely lasts longer than 14 days, and prolonged increases suggest a poor prognosis or the presence of a cyst. Serum lipase may also be elevated in patients with chronic pancreatitis, obstruction of the pancreatic duct and non pancreatic conditions including renal diseases, various abdominal diseases such as acute cholecystitis, intestinal obstruction or infarction, duodenal ulcer, and liver disease, as well as alcoholism & diabetic keto-acidosis & in patients who have undergone endoscopic retrograde cholangiopancreatography. Elevation of serum lipase activity in patients with mumps strongly suggests significant pancreatic as well as salivary gland involvement by the disease.

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SEROLOGY & IMMUNOLOGY REPORT

Test Description	Result	Unit	Biological Reference Ranges
CHIKUNGUNYA ANTIBODIES TEST			
Chikungunia IgG	Negative		
Chikungunia IgM	Negative		

Method : Immunochromatography

INTERPRETATION

1) **Chikungunya** (also known as **chikungunya virus** disease or **chikungunya fever**) is a debilitating, but non-fatal, viral illness that is spread by the bite of infected mosquitoes. It resembles dengue fever.
2) **Chikungunya** is caused by the **chikungunya virus**, which is classified in the family Togaviridae, genus Alphavirus. **Chikungunya** is spread by the bite of an Aedes mosquito, primarily Aedes aegypti. Humans are thought to be the major source, or reservoir, of **chikungunya** virus for mosquitoes.
3) Therefore, the mosquito usually transmits the disease by biting an infected person and then biting someone else. An infected person cannot spread the infection directly to other persons (i.e. it is not a contagious disease). Aedes aegypti mosquitoes bite during the day time.
4) **Epidemics** are sustained by the human-mosquito-human transmission cycle.
5) **Chikungunya** usually starts suddenly with fever, chills, headache, nausea, vomiting, joint pain, and rash. In Swahili, **.chikungunya** means that which contorts or bends up.. This refers to the contorted (or stooped) posture of patients who are afflicted with the severe joint pain (arthritis) which is the most common feature of the disease.
6) Frequently, the infection causes no symptoms, especially in children. While recovery from **chikungunya** is the expected outcome, convalescence can be prolonged and persistent joint pain may require analgesic (pain medication) and long-term anti-inflammatory therapy.
7) Infection appears to confer lasting immunity. The time between the bite of a mosquito carrying **chikungunya virus** and the start of symptoms ranges from 1 to 12 days.

**** End of the report. ****
Please correlate clinically.

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